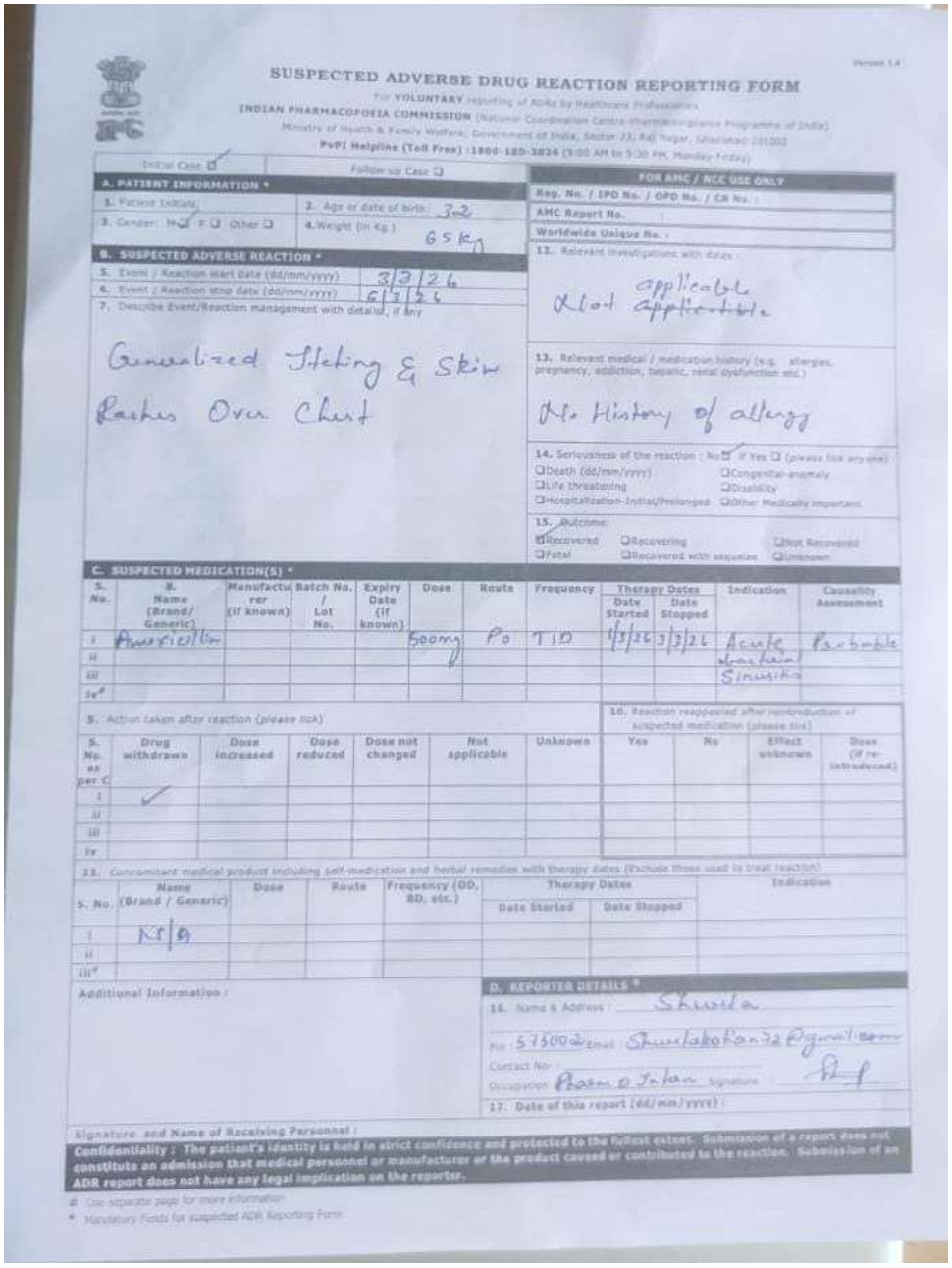
ADR REPORT FORM OF THE GIVEN CASE SCENARIO IN THE TUTORIAL VIDEO



SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For VOLUNTARY reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector 23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) : 1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☑ | Follow up Case ☐

A. PATIENT INFORMATION *

1. Patient Initials:
2. Age or date of birth: 3o2
3. Gender: M ☑ F ☐ Other ☐
4. Weight (in Kg.): 65 Kg

B. SUSPECTED ADVERSE REACTION *

5. Event / Reaction start date (dd/mm/yyyy): 3/3/26
6. Event / Reaction stop date (dd/mm/yyyy): 6/3/26
7. Describe Event/Reaction management with details, if any:

Generalized Itching & Skin Rashes Over Chest

FOR AMC / NCC USE ONLY

Reg. No. / IPD No. / OPD No. / CR No.:
AMC Report No.:
Worldwide Unique No.:

12. Relevant investigations with dates:

Not applicable

13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.):

No History of allergy

14. Seriousness of the reaction : No ☑ If Yes ☐ (please tick any one)
☐Death (dd/mm/yyyy) ☐Congenital anomaly
☐Life threatening ☐Disability
☐Hospitalization-Initial/Prolonged ☐Other Medically important

15. Outcome:
☑Recovered ☐Recovering ☐Not Recovered
☐Fatal ☐Recovered with sequelae ☐Unknown

C. SUSPECTED MEDICATION(S) *

S. No.	B. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication	Causality Assessment
i	Amoxicillin				500mg	PO	TID	1/3/26	3/3/26	Acute bacterial Sinusitis	Probable
ii											
iii											
iv#											

9. Action taken after reaction (please tick)

S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown
i	✓					
ii						
iii						
iv						

10. Reaction reappeared after reintroduction of suspected medication (please tick)

Yes	No	Effect unknown	Dose (if re-introduced)

11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)

S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication
i	NIL						
ii							
iii#							

Additional Information :

D. REPORTER DETAILS *

16. Name & Address: Shweta

Pin: 573002 Email: Shwetabotan72@gmail.com

Contact No:

Occupation: Botan & Intern Signature: Sh

17. Date of this report (dd/mm/yyyy):

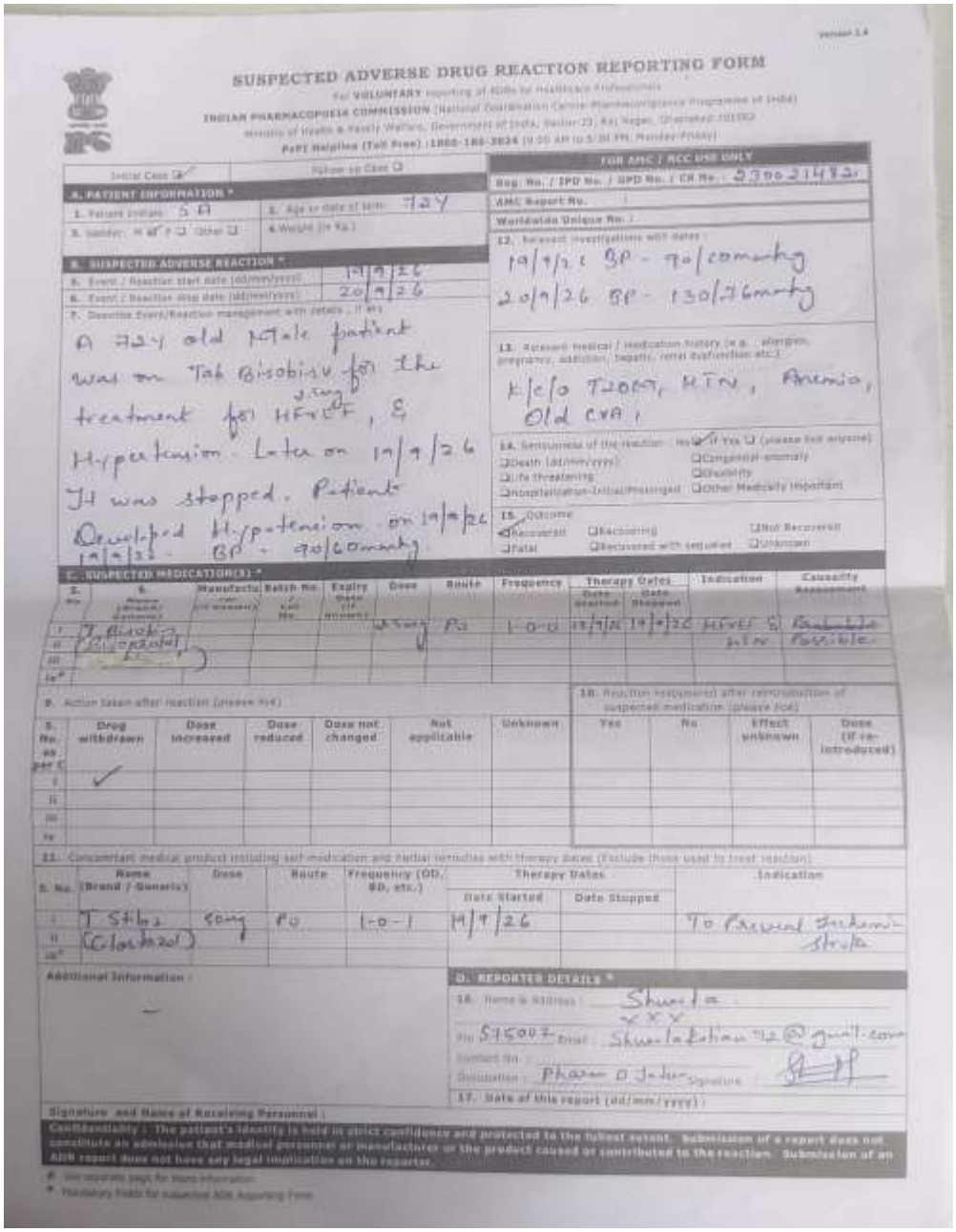
Signature and Name of Receiving Personnel :

Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.

Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

ADR REPORT FORM OF THE CASE SCENARIO I HAVE SELECTED



SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For VOLUNTARY reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) : 1800-180-3024 (9.00 AM to 5.30 PM, Monday-Friday)

Initial Case ✓ | Follow-up Case ☐

FOR AMC / ACC USE ONLY

Hosp. No. / IPD No. / OPD No. / CR No. : 3390 21482

AMC Report No. :

Worldwide Unique No. :

A. PATIENT INFORMATION *

1. Patient initials : S A

2. Age or date of birth : 72Y

3. Gender : M ✓ F ☐ Other ☐

4. Weight (in Kg.)

B. SUSPECTED ADVERSE REACTION *

5. Event / Reaction start date (dd/mm/yyyy) : 19/9/26

6. Event / Reaction stop date (dd/mm/yyyy) : 20/9/26

7. Describe Event/Reaction management with details , if any :

A 72y old female patient was on Tab Bisobis-V 2.5mg for the treatment for HFrEF, & Hypertension. Later on 19/9/26 JH was stopped. Patient Developed Hypotension on 19/9/26 19/9/26 BP - 90/60mmHg

12. Relevant investigations with dates :

19/9/26 BP - 90/60mmHg

20/9/26 BP - 130/76mmHg

13. Relevant medical / medication history (e.g. : allergies, pregnancy, addiction, hepatic, renal dysfunction etc.) :

K/C/O T2DM, HTN, Anemia, Old CVA.

14. Seriousness of the reaction : Yes ✓ If Yes ☐ (please tick anyone)

☐Death (dd/mm/yyyy) ☐Congenital anomaly

☐Life threatening ☐Disability

☐Hospitalization-Initial/Prolonged ☐Other Medically Important

15. Outcome

✓Recovered ☐Recovering ☐Not Recovered

☐Fatal ☐Recovered with sequelae ☐Unknown

C. SUSPECTED MEDICATION(S) *

S. No.	a. Name (Brand / Generic)	Manufacturer (if known)	Batch No. (if known)	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication	Causality Assessment
I	T. Bisob-2.5 (Bisoprolol)				2.5mg	PO	1-0-0	19/9/26	19/9/26	HFrEF & HTN	Probable Possible
II											
III											
IV*											

9. Action taken after reaction (please tick)

10. Reaction reappeared after reintroduction of suspected medication (please tick)

S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)
I	✓									
II										
III										
IV										

11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)

S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication
I	T Stbz (Clopidogrel)	50mg	PO	1-0-1	19/9/26		To Prevent Ischemic Stroke
II							
III*							

Additional Information :

–

D. REPORTER DETAILS *

16. Name & Address : Shweta XXX

Pin 575007 Email : Shwetakotian72@gmail.com

Contact No. :

Occupation : Pharm D Intern Signature :

17. Date of this report (dd/mm/yyyy) :

Signature and Name of Receiving Personnel :

Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.

Use separate page for more information

* Mandatory fields for suspected ADR Reporting Form

CASE SCENARIO :

Patient name : S A

Age: 72 year

Gender: Male

Allergy status : No known allergy

Past Medical history : Hypertension, Type 2 Diabetic mellitus, Old CVA

Present illness : Recurrent ais - right mca-pca watershed infarct + right parietal infarct with left hemiplegia, old right ica distal occlusion (only on tab stiloz),dvt left ll, ?sepsis with septic shock, ckd, ild - usual interstitial pneumonia, type 2 dm (hba1c: 7.7%),systemic hypertension, hfref (ef: 34%), Anemia of chronic disease, old pulmonary tb (30 years back)

Past medication history : Tab Stiloz 50 1-0-1
Tab Atorva 0-0-1
Tab Amlodac AT (Amlodipine & Atenolol)
Tab Dytor 10 1-0-0
Tab Cospiaq Sm (Empa+Sita+MF) 25/100/100mg 1/2-0-1/2

A 72 Year old male patient was Started on 18/9/2026 Tab Bisobis (Bisoprolol) 2.5mg For HFrEF and Hypertension. Later on 19/9/2026 he developed Hypotension. The drug was stopped. On 20/09/2026 Blood pressure came to normal 130/76mmhg.

Laboratory data :

Physical examination: on 19/9/2026 Blood pressure: 90/60 mmhg ,

20/9/2026 Blood presuure:130/76mmhg

Management:

Tab Bisobis 2.5 mg was discontinued due to hypotension. Noradrenaline support was documented for hypotension. Blood pressure monitoring was advised every 2 hours.

On 20/09/2026 Blood pressure came to normal 130/76mmhg.